

EVIDENCE AXIS

Complimentary Competitive Intelligence Brief

AISEL HEALTH

UK entry: procurement route and mental-health workflow pressure

Public evidence checked 27 Aug 2026

The first UK question may not be feature comparison. It may be route-to-market proof: which assurance signals get Aisel into institutional procurement, and which existing NHS mental-health platforms can add documentation faster than a new psychiatry OS can win distribution?

This brief does not assess internal compliance status. It compares public evidence, public proof, and observable market movement.

Recipient route: Founder / CEO
Package: Executive Brief + Evidence Appendix

Why we looked

Aisel just raised to scale a psychiatry-specific operating system into the UK.

Trigger

Aisel announced a EUR 1.7M pre-seed round to scale its psychiatry OS into the UK. Its public product now spans intake, scribe, EHR sync, and heavy psychiatric documentation. [A1, A4]

What changed outside Aisel

NHS England updated ambient-scribing guidance and maintains a supplier registry for AVT products. Several listed suppliers are already visible in the UK documentation market. [N1-N3]

Why this matters

For a UK institutional motion, the proof layer may matter as much as product capability. Buyers can see assurance evidence for listed AVT suppliers before they see Aisel in the same public path.

DECISION QUESTION

Is Aisel entering the UK as a private-clinic psychiatry OS first, or as an NHS/institutional supplier that must win procurement confidence immediately?

The answer changes the competitive set, the proof buyers expect, and the first follow-up work Aisel should prioritize.

Scope boundary: public evidence only; no internal Aisel roadmap, regulatory file, procurement pipeline, or customer conversations reviewed.

The public UK route is tighter than the site suggests

Finding 1 - procurement route signal

FINDING

Aisel publicly targets institutional buyers, including NHS and EU healthcare systems, but the reviewed public evidence does not show Aisel on the NHS AVT supplier registry.

Aisel public position

Institutional page: NHS/EU systems, procurement support, UK/EU hosting, SSO, audit logs, data-residency controls.
Product page: Aisel Scribe, in-person/video consultations, EHR sync. [A1, A2]

Registry public signal

NHS AVT registry lists 27 suppliers that provided evidence of meeting criteria; Heidi, Tortus, Tandem and others are listed. Aisel is not listed under the reviewed name. [N1]

Buyer implication

Find a Tender says the registry is not a procurement mechanism, but is used to support local due diligence and route-to-market decisions. [N3]

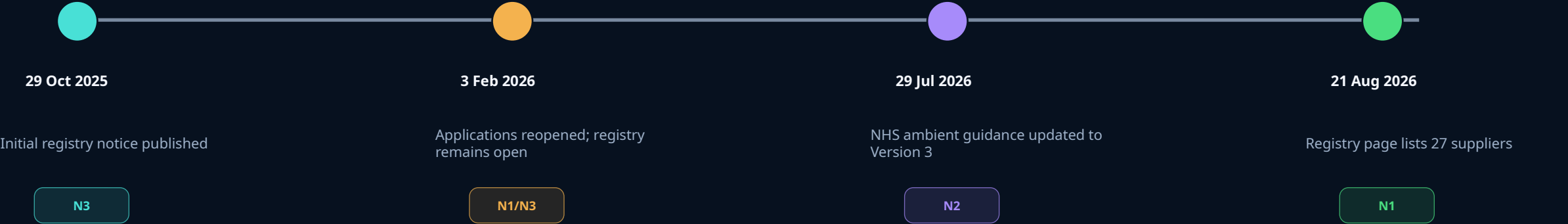
Guardrail: this does not say Aisel lacks DTAC, DCB0129, MHRA registration, or registry work in progress. Public evidence only shows that these signals were not visible in the reviewed public path.

Why registry visibility matters now

The public NHS route changed recently, not in the distant past.

NHS England says the AVT registry supports safe and effective scaling of ambient voice technology and helps local procurement and assurance. The registry page was last edited 21 Aug 2026. Applications remain open. [N1]

Decision signal: if NHS institutional procurement is a near-term path, registry readiness is not a website copy issue. It is a commercial sequencing issue.



The threat is not only standalone scribes

Finding 2 - existing NHS mental-health workflow distribution is moving into documentation.

FINDING

Limbic is already visible inside NHS mental-health workflows, and in June 2026 it added a TORTUS-powered medical AI scribe inside Limbic Care.

Existing mental-health foothold

Oxford Health NHS describes Limbic as an AI chatbot route for Talking Therapies access. A UK Parliament answer says NHS Talking Therapies offers tools such as Limbic Access and Wysa. [L2, L3]

Move into documentation

Limbic announced it was embedding a TORTUS-powered medical AI scribe into Limbic Care for NHS mental-health services. [L1]

Registry bridge

TORTUS is one of the 27 suppliers listed on the NHS AVT registry. [N1]

Implication: Aisel may face a distribution-path competitor, not just a product-feature competitor: an existing mental-health platform can attach a registered AVT supplier and enter documentation from inside the workflow.

Aisel still has a sharper wedge

The defensible claim may be psychiatry memory and high-stakes documents - not “AI scribe”.

WHERE AISEL IS STRONGEST IN PUBLIC EVIDENCE



Recommended frame for Aisel: not “another AI note taker”, and not just “psychiatry-specific scribe”. The public evidence supports a more defensible wedge: psychiatric memory plus source-linked, high-stakes documentation that can survive review.

This is an inference from the public product evidence and should be validated against Aisel’s actual UK sales conversations.

Competitor roles: keep them separate

Do not collapse different evidence roles into a fake single competitive set.

Heidi

UK AVT benchmark

Shows public NHS-specific assurance signals; mental-health pages exist, but used here mainly as procurement-proof benchmark. [H1, H2]

Limbic + TORTUS

Adjacent UK mental-health workflow threat

Mental-health workflow foothold plus scribe added through a registry-listed AVT partner. [L1-L3, N1]

JotPsych

Global psychiatry-platform drift

US/Canada/AU/NZ evidence; current product appears broader than public coverage suggests. UK presence not established. [J1]

Guardrail: Heidi is not presented as a psychiatry competitor. JotPsych is not presented as a UK competitor. Limbic/TORTUS is an adjacent UK mental-health workflow threat, not an identical psychiatry OS.

Decision matrix

What Aisel may need to decide before the UK push.

1 Route

Is the near-term UK motion private clinics first, or institutional/NHS procurement?

2 Proof

If NHS is near-term, should registry, DTAC, MHRA, DCB0129, and public assurance be treated as commercial dependencies?

3 Wedge

Should the first UK story center on psychiatry memory and defensible documents rather than ambient scribe speed?

4 Response

Who gets the first battlecard: Heidi, Limbic/TORTUS, or JotPsych? Each plays a different role.

Suggested watch action

Before more UK outreach, map the procurement path against the actual go-to-market plan. If NHS institutional entry is near-term, the registry and assurance story may deserve roadmap priority before another feature.

What a paid sprint would answer

The Gift surfaces the public evidence; the paid work needs Aisel’s actual context.

Competitive Decision Sprint

Confirm the target buyer path, rank Heidi / Limbic-TORTUS / JotPsych by actual sales context, and define what Aisel should prove first for UK buyers.

Response Sprint

Turn the evidence into UK procurement collateral, founder-level narrative, sales objections, and competitor-specific response logic.

Radar

Monitor AVT registry changes, Limbic/TORTUS rollout, Heidi mental-health positioning, and psychiatry-platform expansion signals.

IF USEFUL

Reply with the UK buyer path you want tested first: private clinics, institutional/NHS, or both.

About Evidence Axis: independent competitive intelligence and decision support. Public evidence is separated into Fact / Inference / Unknown, with source boundaries disclosed.